

1. Administrative & Study Identification

Full Study Title: A Randomized, Controlled, Open Label, Phase III Study Evaluating the Efficacy and Safety of JDQ443 Versus Docetaxel in Previously Treated Subjects With Locally Advanced or Metastatic KRAS G12C Mutant Non-small Cell Lung Cancer **Short Title/Acronym:** KonTRASt-02 **Protocol Number:** CJDQ443B12301 **NCT Number:** NCT05132075 **Posting ID:** 61400814159910334 **Sponsor Name:** Novartis (Novartis Pharmaceuticals) **Investigational Product:** JDQ443 / Docetaxel **Phase of Development:** Phase III **Trial Status:** Active, not recruiting (ACTIVE_NOT_RECRUITING) **Medical Monitor:** Novartis Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate progression-free survival (PFS) as assessed by a Blinded Independent Review Committee (BIRC) in participants receiving JDQ443 compared to docetaxel. **Secondary Objectives:** To evaluate overall survival (OS), objective response rate (ORR), safety, and health-related quality of life (using EORTC-QLQ-C30, EORTC-QLQ-LC13, and EQ-5D-5L questionnaires).

2.2 Background & Rationale Kirsten rat sarcoma viral oncogene homolog (KRAS) is a frequent mutated oncogene in non-small cell lung cancer (NSCLC), with the KRAS G12C variant present in ~13% of non-squamous NSCLC patients. JDQ443 is a potent, structurally novel, selective KRAS G12C inhibitor. For patients with advanced NSCLC who progress following first-line immunotherapy or doublet platinum-based chemotherapy, single-agent docetaxel remains a standard option, though it typically offers modest activity and is poorly tolerated. This study aims to provide an alternative, more effective targeted treatment option to improve patient outcomes.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Docetaxel) **Blinding:** Open-label **Randomization:** 1:1, stratified by ECOG performance status (0 vs 1 and 2) and prior therapy (combined vs sequential)

3.2 Planned Enrollment & Duration Target \$N\$: Targeted Enrollment is 95 (Protocol estimates approximately 360 participants to be randomized) **Number of Sites:** Global/Multicenter **Estimated Study**

Start: Mid-2022 **Estimated Primary Completion:** Up to ~33 months follow-up

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Participant has histologically confirmed locally advanced/metastatic (stage IIIB/IIIC or IV) NSCLC. 2. Participant has a KRAS G12C mutation present in tumor tissue or plasma prior to enrollment, as determined by a Novartis-designated central laboratory or accepted local tests. 3. Participant has received one prior platinum-based chemotherapy regimen and one prior immune checkpoint inhibitor therapy for locally advanced or metastatic disease. 4. Participant has at least 1 evaluable lesion (measurable or non-measurable) by RECIST 1.1 at the screening visit.

4.2 Exclusion Criteria 1. Participant has previously received docetaxel (except if received in neoadjuvant or adjuvant setting with no progression within 12 months after the end of treatment), or any other KRAS G12C inhibitor. 2. Participant has an EGFR-sensitizing mutation and/or ALK rearrangement by local laboratory testing. 3. Participant has known active central nervous system (CNS) metastases and/or carcinomatous meningitis. 4. Participant has a history of interstitial lung disease or pneumonitis grade > 1.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: * **Investigational Arm:** JDQ443 200 mg twice daily continuously. * **Comparator Arm:** Docetaxel (Trade Names: Docefrez, Taxotere; ATC Code: L01CD02) 75 mg/m² intravenous once every 21 days. **Route of Administration:** Oral (JDQ443) / IV (Docetaxel) **Duration of Treatment:** Until disease progression, unacceptable toxicity, or patient withdrawal.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care for managing chemotherapy-related toxicities (e.g., anti-emetics) as determined by the investigator. **Prohibited:** Other systemic anti-cancer therapies, concurrent KRAS G12C inhibitors, or additional investigational agents.

6. Study Timeline & Visit Schedule

| Visit ID | Window (Days) | Key Activities/Procedures | | :---
| :--- | :--- | | **Screening** | Day -28 to 0 | Informed Consent, Medical History, Tumor Mutational Testing (KRAS G12C), Baseline RECIST 1.1 Scan | | **Baseline** | Day 0 | Randomization, First Dose Administration, PRO Questionnaires | | **Interim** | Every 3 Weeks | Safety Labs, Treatment Administration (Docetaxel arm), Efficacy/

AE Monitoring | | **End of Study** | Up to 33 Months | Final Vital Signs, BIRC Tumor Assessment, Exit Interview, Survival Follow-up |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Progression-Free Survival (PFS) as assessed by a Blinded Independent Review Committee (BIRC).
Measurement Method: Radiological tumor assessments per RECIST version 1.1.

7.2 Secondary & Exploratory Endpoints * Overall Survival (OS) * Objective Response Rate (ORR) * Disease Control Rate (DCR) * Changes from baseline in health-related quality of life assessments: EORTC-QLQ-C30, EORTC-QLQ-LC13, and EQ-5D-5L.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection via regular investigator assessments, laboratory panels, and patient-reported outcomes (PROs). **Serious Adverse Events (SAEs):** Reported within 24 hours of site awareness. **Data Monitoring Committee (DMC):** Independent Data Monitoring Committee (IDMC) to review safety and efficacy signals.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy endpoints. Safety Set for toxicity and adverse events evaluations. **Sample Size Justification:** Target enrollment powered to detect a statistically significant difference in PFS between JDQ443 and docetaxel. **Handling of Missing Data:** Standard survival analysis principles (e.g., right-censoring for PFS/OS) and mixed models for PRO endpoints.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Regular onsite and remote monitoring to assure protocol adherence. **Audit Trail:** Validated eCRF systems, BIRC-audited radiological assessments.

11. Study Identifiers & Governance

Primary ID: CJDQ443B12301 **Registry Number:** NCT05132075 **Posting ID:** 61400814159910334 **Sponsor/Lead Organization:** Novartis **Study**

Title: KonTRASt-02 (Study of JDQ443 in Comparison With Docetaxel in Participants With Locally Advanced or Metastatic KRAS G12C Mutant Non-small Cell Lung Cancer) **Official Regulatory Title:** A Randomized, Controlled, Open Label, Phase III Study Evaluating the Efficacy and Safety of JDQ443 Versus Docetaxel in Previously Treated Subjects With Locally Advanced or Metastatic KRAS G12C Mutant Non-small Cell Lung Cancer

12. Clinical Framework (NSCLC Specific)

Primary Condition: Non-Small Cell Lung Cancer (NSCLC) / Metastatic non-small cell lung cancer **Preferred UMLS Name:** Non-Small Cell Lung Carcinoma with KRAS Mutation **Diagnosis Threshold:** Locally advanced or metastatic (Stage IIIB/IIIC or IV) with confirmed KRAS G12C mutation **Medical Methodology:** Targeted therapy (KRAS G12C inhibitor) versus cytotoxic chemotherapy **Optimization Target:** Progression-free and overall survival prolongation

13. Study Procedures & Methodology

Management Pathway: Standard oncological pathway for relapsed/refractory advanced NSCLC **Education Protocol (HCP):** Investigator training for BIRC/RECIST 1.1 requirements and AE management **Education Protocol (Patient):** Counseling on targeted therapy vs. chemotherapy side effects and PRO questionnaire completion **Quality Audit Mechanism:** Blinded Independent Review Committee (BIRC) assessment of radiographic images

14. Observation & Follow-Up Schedule

Total Duration: Follow-up for up to ~33 months **Onsite Visit Cadence:** Every 3 weeks (aligned with docetaxel cycle dosing) **Remote Monitoring Frequency:** Between cycles and during long-term survival follow-up **Checklist Review Interval:** Routine imaging scans every 6 to 8 weeks for PFS assessment

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				